BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

Check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver	Baseline blood count, kidney and liver profile	Review results before treatment	Blood count, kidney and liver tests before each cycle	Repeat sooner if symptoms or results need it	If treatment is delayed for low blood counts, repeat the blood count weekly until the team confirms recovery.
Magnesium, calcium and potassium	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Bowel symptoms and hydration	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day: act at the first liquid stool using your supplied rescue plan and electrolyte fluids; contact the team early.	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review
Fluorouracil safety and line	Confirm the DPD test and safe line/pump arrangements	Check mouth, bowel and pump symptoms early	Review line, pump and previous-cycle toxicity	Report chest pain or unexpectedly severe early side effects immediately	Confirm disconnection and ongoing line care
Cancer response and treatment choices	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00328 v7 · official regimen · EMA: Erbitux product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Weekly cetuximab + FOLFIRI: treatment and checks

Advanced / metastatic colorectal cancer

Cetuximab on Day 1 and Day 8. Day 1 FOLFIRI includes irinotecan, folinic acid and fluorouracil, followed by a 46-hour fluorouracil infusion with planned disconnection around Day 3. Repeat the reference cycle every 14 days while beneficial and tolerable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver NCCP 00328	Baseline blood count, kidney and liver profile	Review results before treatment	Blood count, kidney and liver tests before each cycle	Repeat sooner if symptoms or results need it	If treatment is delayed for low blood counts, repeat the blood count weekly until the team confirms recovery.
Magnesium, calcium and potassium NCCP 00328	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
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Bowel symptoms and hydration NCCP 00328	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day: act at the first liquid stool using your supplied rescue plan and electrolyte fluids; contact the team early.	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review
Fluorouracil safety and line NCCP 00328	Confirm the DPD test and safe line/pump arrangements	Check mouth, bowel and pump symptoms early	Review line, pump and previous-cycle toxicity	Report chest pain or unexpectedly severe early side effects immediately	Confirm disconnection and ongoing line care
Cancer response and treatment choices NCCP 00328	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

What each check is for

1

A 14-day reference cycle

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00328

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.34